

CONSULTATION & COLLABORATION

Clinical Supervision Agreement & Session Log

A clear contract between supervisor and supervisee covering roles, availability, emergencies, recording, evaluation and documentation, with a log and a quarterly review form.

AUDIENCE
Supervisors · Supervisees

FORMAT
Agreement + log +
evaluation

VERSION
1.0 · Sept 2026

TIME TO ADAPT
30–45 minutes

WHAT IT IS

A written agreement between a supervisor and supervisee, with a session log and a quarterly evaluation form.

USE IT WHEN

You are starting licensure-track, postdoctoral or consultative supervision, or the current arrangement is unwritten.

CUSTOMIZE FIRST

Your board's supervisor eligibility, hour and modality rules (including telesupervision), and any employer or training-program requirements.

Why a written agreement

Supervision combines teaching, oversight and evaluation, and it carries legal and ethical responsibility for patients the supervisor may never meet. Misunderstandings about availability, emergencies, recording and evaluation are the most common source of conflict. The APA Guidelines for Clinical Supervision in Health Service Psychology (2015) recommend an explicit supervision contract. The APA Ethics Code addresses delegation of work (Standard 2.05) and assessing supervisee performance (Standard 7.06).

CHECK YOUR BOARD FIRST

Boards set who may supervise, what hours count toward licensure, how much must be face to face or live, whether telesupervision is allowed, and what documentation is required. This template is a starting point and does not override your board's rules.

Part A: Supervision Agreement

1. Parties and type of supervision

Supervisor name, credential, license and state

Contact

Supervisee name, credential, license (if any) and state

Contact

Setting / practice

Start date

Review date

-
- ☐ Licensure or postdoctoral supervision (hours to be reported to the board)
- ☐ Consultative supervision for a licensed clinician
- ☐ Supervision of an unlicensed professional working under the supervisor's license
- ☐ Other: **[describe]**

2. Supervisor's responsibilities

- Provide regular, scheduled supervision of at least **[1]** hour per **[week]** and be reasonably available between sessions.
- Review the supervisee's caseload and clinical work with appropriate depth for their competence, and set the level of independence accordingly.
- Provide timely, specific feedback and a written evaluation at least **[quarterly]**, with the criteria shared in advance.
- Be aware of the supervisee's patients, risk and documentation, and accept the responsibility that the law and their license attach to supervisors.
- Model ethical, culturally responsive practice and address issues of diversity and power in supervision.
- Keep records of supervision and report hours accurately.

3. Supervisee's responsibilities

- Prepare for and attend supervision, and bring cases, recordings or notes as agreed.
- Disclose errors, ruptures, boundary concerns and any incident promptly, including between sessions when risk is involved.
- Work within their competence and the scope of the supervisor's license and the board rules.
- Tell patients they are being supervised and by whom, and obtain consent as required (see section 6).
- Keep documentation complete and on time, and follow practice policies.

4. Structure

Frequency and length

Format (in person / video / phone)

Individual / group

Content (case review, live observation, recordings, readings)

Telesupervision is used only if allowed by the board and the setting, over a HIPAA-compliant platform, with the supervisee's location known to the supervisor. Sessions are not recorded or transcribed by AI tools unless both parties and any patient involved have agreed in writing.

5. Emergencies and availability

If a patient is at risk or an urgent legal, ethical or safety issue arises, the supervisee contacts the supervisor at **[phone]** as soon as possible. If the supervisor is unreachable, the supervisee contacts **[backup supervisor]** at **[phone]**. The supervisee follows the practice's crisis procedures and 988 and 911 as appropriate. The supervisor's planned absences are announced **[two weeks]** in advance with a named backup.

6. Patient consent and disclosure

Patients are told, before or at the first session, that the supervisee is [an unlicensed clinician / a postdoctoral resident] working under the supervision of [supervisor name, credential] and that the supervisor may review their records and, if applicable, recordings. Written informed consent covers supervision, and recording if used.

7. Recording and documentation

- Recordings: [only with patient written consent; stored on encrypted devices; deleted after review / retained until (date)].
- Supervision log: both parties sign each entry. The supervisee keeps the log and the supervisor keeps a copy.
- The supervisor cosigns notes and reports as required by law, the payer, or the practice.

8. Evaluation

Evaluation is ongoing and criterion-based, using the competency areas on the Quarterly Review form and any program or board form. Supervisor and supervisee each complete the form and discuss it. If concerns arise, the supervisor names them early, sets specific expectations and a timeline, and documents the plan.

9. Fees

[No fee / Supervisee pays \$ per hour / Supervisor is compensated by (employer or practice)]. Any arrangement complies with board rules on fees for supervision and does not depend on referrals.

10. Confidentiality and its limits

Supervision content is confidential, except: patient safety, legal or ethical reporting, communications required with the board, the training program or employer, and evaluation results. Supervisees' personal disclosures are limited to what is relevant to professional functioning (see APA Ethics Code Standard 7.04).

11. Concerns, conflicts and ending

Concerns are raised first with each other, then with [program director / a neutral consultant] if unresolved. Either party may end the arrangement with [30] days' notice, or immediately where patient safety requires. Hours completed are documented and verified. The supervisor helps plan transition of the supervisee's patients.

Supervisor signature

Date

Supervisee signature

Date

Part B: Supervision session log

Date	Length	Format	Type (individual / group / live)	Topics and cases (by ID)	Competenc ies	Action items	Initial s

Running total of hours this period: . Cumulative total toward licensure: .

Part C: Quarterly review

Rate each area from 1 (not yet at expected level) to 5 (exceeds expected level for training stage). Add examples.

Competency area	Supervisor rating	Supervisee rating	Examples and goals
Assessment and diagnostic formulation			
Case conceptualization and treatment planning			
Therapeutic relationship and interventions			
Risk assessment and crisis management			
Ethical and legal decision-making			
Cultural humility and responsiveness			
Documentation quality and timeliness			
Use of supervision and openness to feedback			
Professional conduct and self-care			

Strengths

Priority development goals for next quarter

Supervisor signature and date

Supervisee signature and date

IN PSYALLIANCE

The Supervision module matches supervisors and supervisees by state, specialism and availability. Agree terms here, then track hours in your personal library.

Sources and further reading

- APA, Guidelines for Clinical Supervision in Health Service Psychology (2015).
- APA Ethics Code, Standards 2.05, 7.04 and 7.06 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- Your state psychology board's supervision rules and, for postdoctoral hours, your jurisdiction's licensure requirements (ASPPB directory, [asppb.net](https://www.asppb.net)).

ABOUT THIS DOCUMENT

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